Media Information

From: Main Street Radiology To: A Page: 1/2 Date: 9/16/2025 9:16:37 AM

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.: HELEN HSIEH, MD
.: 133-47 SANFORD AVE. SUITE 1B
FLUSHING, NY [redacted]

Referrer Phone: (718)661-6630
Name: WEN MEI I YEH
Phone: [redacted]
Gender: Female
DOB: [redacted]
Patient [redacted] [redacted]
Exam Date: 8/26/2025 09:18 AM

Acc #: 10000022
Secondary ID: 191758
Exam Name: [redacted] 77063

Right Mammogram:

Clinical information: 75-year-old female for diagnostic mammography. Follow-up of a probably benign right breast asymmetry.

Breast Cancer Risk:
NCI 5 year:2.8%
NCI Lifetime:7.5%

Last reported clinical breast exam: Within the past year

Comparison:

3D digital CC and MLO views of the right breast were obtained. Reconstructed CC and MLO views were obtained. Deep learning algorithms (AI) were applied to mammography images and DBT services as a concurrent reading aid. Spot compression imaging was performed.

The breast tissue is heterogeneously dense, which may obscure small masses. There is a persistent irregular focal asymmetry in the upper outer quadrant. A focal asymmetry in the subareolar region effaces with spot compression imaging is grossly stable compared to September 2021 accounting for differences in technique, benign.

Right Breast Ultrasound

Comparison: [redacted] through [redacted]

Sonographic evaluation of the right breast in its entirety was performed, including each of the four quadrants, the retroareolar region, and the axilla, in clockwise fashion.

-11:00, 8 cm from the nipple, there is a 0.6 cm irregular hypoechoic mass likely corresponding to mammographic finding.

Continued...

Main Street Radiology

Name: [redacted]
Date of Birth: [redacted]

-10:00, 2 cm from nipple, there is a 0.8 cm elongated cyst, benign.
-Axilla, no [redacted]

IMPRESSION:

Persistent irregular focal asymmetry in the right upper outer quadrant with likely indeterminate sonographic correlate at 11:00, 8 cm from the nipple. Ultrasound-guided core needle biopsy with clip placement and post procedure mammogram for correlation is recommended.

BI-RADS CATEGORY: 4 - Suspicious Abnormality
Recommendation: Right breast ultrasound-guided needle biopsy with clip placement and post procedure mammogram.

Recommendation: Ultrasound Biopsy

The above findings and recommendations were relayed to the patient [redacted] and discussed with Caroline from Dr. Hsieh's office on [redacted] 11:23 AM.

A letter will been sent to the patient [redacted]

Approximately 10% of breast carcinomas are not radiographically detectable. A negative report should not delay biopsy if a clinically suspicious mass is present. Dense breasts may obscure an underlying neoplasm.

Patient [redacted] a target due date for their next mammogram.

BIRADS 1: Negative
BIRADS 2: Benign
BIRADS 3: Probably Benign
BIRADS 4: Suspicious Abnormality - Biopsy should be considered
BIRADS 5: Highly Suspicious of Malignancy
BIRADS 6: Known Malignancy
BIRADS 0: Incomplete - Need Additional Imaging Evaluation and/or Prior Mammograms for Comparison

The New York State Department of Health requires us to indicate the date of the patient's last clinical breast examination.

Electronically signed by: Allison Rubin MD [redacted] 11:24 AM
Workstation: NYPQARUBIN

Report Electronically Signed by: Allison Rubin MD
Report Electronically Signed on: 8/26/2025 11:24 AM

Document Information

Misc Administration: Outside Radiology, Breast

MA 3D DIGITALDIAGNOSTIC MAMMO (RIGHT BREAST) [redacted] 00:00

Attached To:

OUTSIDE RADIOLOGY ORDER [redacted]

Orders Only on [redacted] with Historical Provider, [redacted]

Source Information

Priscilla P. Hazel | Msk Pas